



Health Claims Forum

**Building on the
Basics**

12.03.25

Royal College of Psychiatrists

Agenda for the day



HCF chair intro; (10.30 – 10.45) – Kieran Tarrant (SCOR)

Sales Distribution (10.45-11.30) – Freddie Huang (SCOR)

UW (11.30 – 12.30) – Leighton Freeman (PLRe)

Lunch (12.30-13.30)

Misrep (13.30 – 14.15)

Retro Underwriting (14.15 – 14.45) – Jen Taylor-Hill (PLRe)

Break 14.45 (25 mins)

TCF Calls (45mins) – 15.10 – 16.15 – Kieran Tarrant (SCOR) and Jen Taylor-Hill (PLRe)

Chair Closing Remarks 16.15 (15mins) - Kieran Tarrant (SCOR)

WHAT DO ATTENDEES SAY ABOUT LUCID?

'This was my first LUCID event, and I'm absolutely determined to ensure it's not my last. I thought it was fantastic from registration to close; it was seamless.'

'Wonderful conference and atmosphere. Well done and look forward to 2025!'

'I loved the detailed presentations, the clear and efficiently run programme, the detail in the app and the chance to meet and chat with others.'

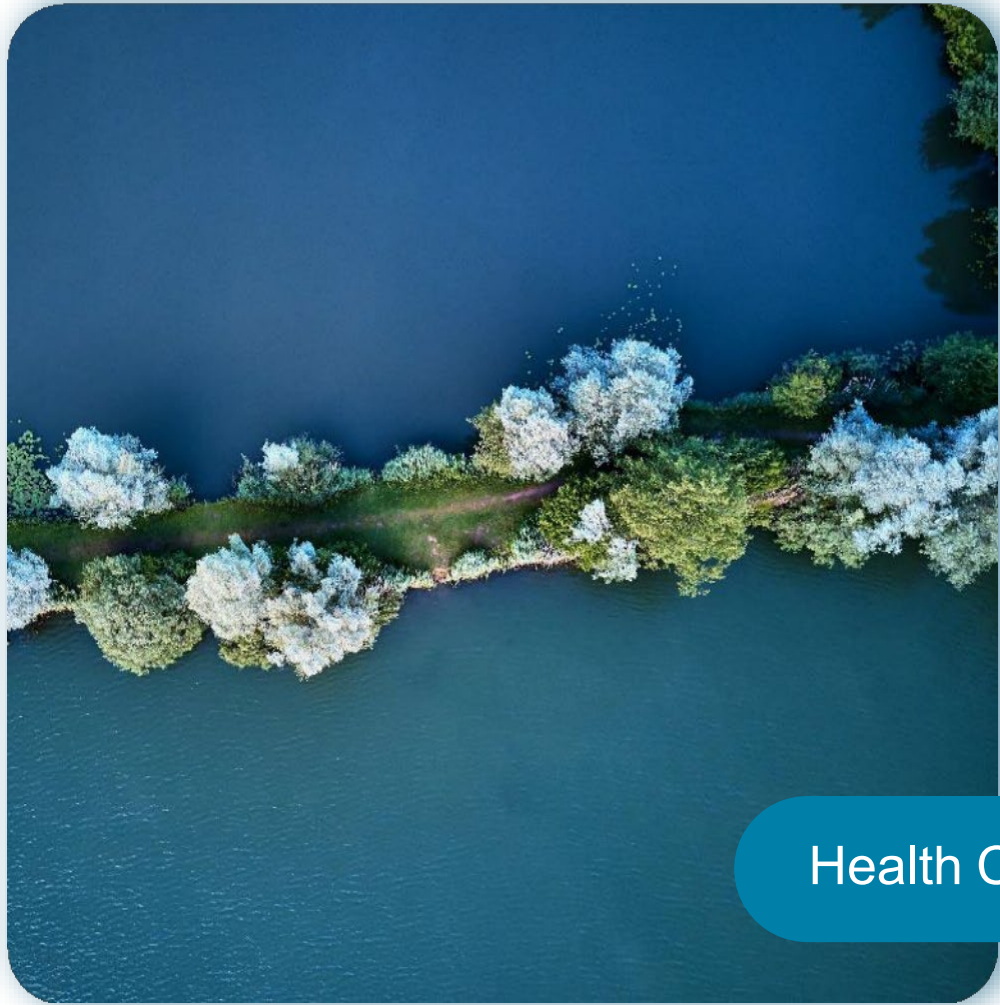
'I loved the diversity of subjects...it's all great education to help us when dealing with our customers and when designing new products.'



LIFE UNDERWRITING, CLAIMS AND INSURANCE DOCTORS

A collaborative conference organised by
representatives from all the UK & Ireland's industry clubs





Life Insurance Distribution

Freddie Huang

Innovation Actuary, SCOR

Health Claims Forum 2025

About Freddie

- **Innovation Actuary at SCOR**
- **Extensive distribution-related expertise from both insurer and reinsurer perspectives**
- **Building and maintaining strong relationship with distribution managers**
- **Conducting Distribution Quality Management (DQM) exercises**

Today's agenda

1 Introduction to Distribution

2 Different Types of Distribution

3 Differences in the Sales Process

4 Case Study

5 Current Industry Practices

6 Q&A

Introduction to Distribution

Insurance Customer Journey



Definition of Distribution

Definition by FCA:

- The activities of advising on, proposing, or carrying out other work preparatory to the conclusion of contracts of insurance, concluding such contracts, or assisting in the administration and performance of such contracts, particularly in the event of a claim
- Provision of information concerning one or more insurance contracts in accordance with criteria selected by customers through a website or other media
- Compilation of an insurance product ranking list, including price and product comparison
- A discount on the price of an insurance contract, when the customer is able to directly or indirectly conclude an insurance contract using a website or other media

Different Types of Distribution

Different types of distribution



Intermediated

1. Traditional face-to-face
2. Telephony advised
3. Telephony non-advised



Direct



Bancassurance



Aggregators



Affinity Partnerships

Different types of distribution

Intermediated

Insurance brokers and financial advisers who offer underwriting and policy application services to customers

Traditional face-to-face: provide advice and sell insurance products through in-person meetings. They typically offer personalized service and detailed consultations to help customers choose the right insurance products

Telephony Advised: These intermediaries offer advice over the phone, guiding customers through the process of selecting suitable insurance products. They provide personalized recommendations based on the customer's needs and circumstances

Telephony Non-Advised: These intermediaries sell insurance products over the phone without providing specific advice. Customers are given information about the products, but the decision-making is left entirely to them



Different types of distribution

Direct

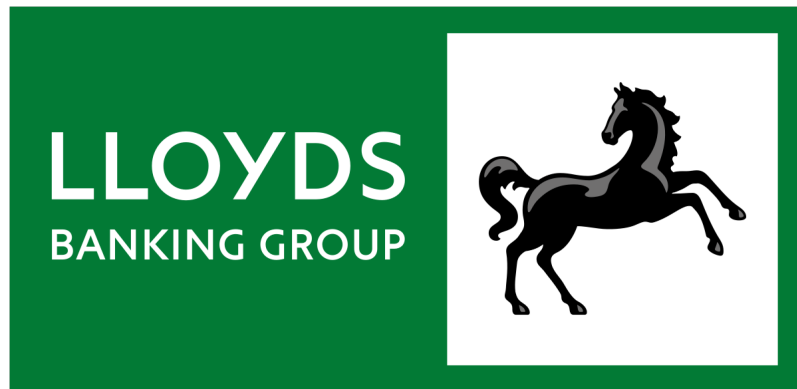
Insurers sell policies directly to consumers through their own sales teams, online platforms, or call centres



Different types of distribution

Bancassurance

Banks sell insurance products, typically through partnerships with insurers



NatWest



Different types of distribution

Aggregators

Price comparison websites that allow customers to compare and purchase insurance products online



MONEYSUPERMARKET

Different types of distribution

Affinity Partnerships

Collaborations with non-insurance organizations like retailers, trade associations, and membership organisations



M&S BANK



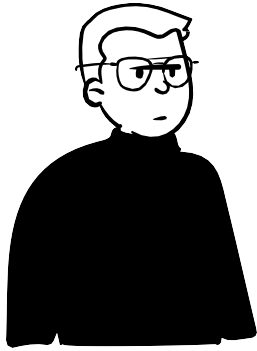
Differences in the Sales Process

Differences in Sales Process

	 Traditional face-to-face	 Telephony Advised	 Telephony Non-advised	 Direct	 Aggregators	 Bancassurance	 Affinity partnership
Price sensitivity							
Level of medical evidence							
Level of description							
Regulation Stringency							
Commission							
Lapse							
Efficiency							

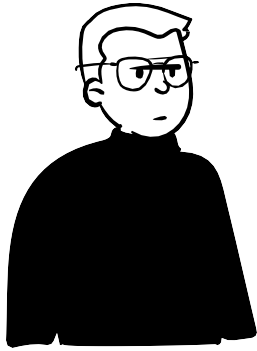
Differences in Sales Process

Matching Exercise



Differences in Sales Process

Matching Exercise



They can choose any distributor!



Traditional
Face-to-Face



Telephony
Advised



Telephony
Non-advised



Direct



Bancassurance

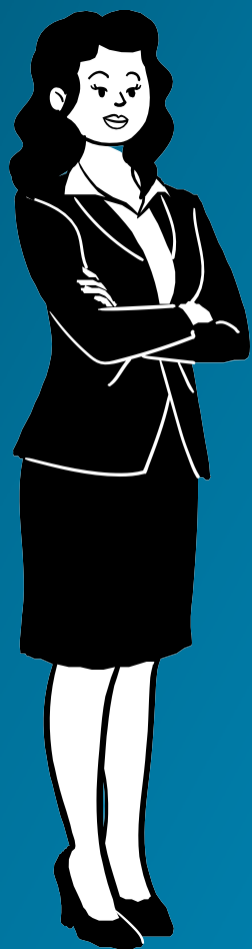


Aggregators



Affinity
Partnerships

Case Study



Meet Sarah, a 35-year-old office worker in London, decides to buy CI insurance. She explores three different distribution channels:

1. Financial adviser A
2. Financial adviser B
3. Comparison website

Scenario

1

- Sarah meets with a telephony adviser.
- The adviser conducts a detailed needs analysis and **explains CI coverage in depth.**
- Sarah completes a comprehensive medical questionnaire with the IFA's guidance.
- The adviser reviews Sarah's answers and asks **follow-up questions.**
- Sarah Discloses a history of migraines and occasional back pain. The adviser explains the relevance of this information, and it's included in the application.

Scenario

2

- Sarah consults a different adviser who seems eager to complete the sale quickly
- The adviser suggests **omitting minor health issues** to save time and potentially get a **lower premium**
- Sarah hesitates but follows the adviser's advice, not mentioning her migraines or back pain

Scenario

3

- Sarah compares policies on a price comparison website
- She completes a brief online questionnaire focusing on **key health issues**
- The process is quick, with **minimal guidance** on question interpretation

Two years later...

Sarah is diagnosed with a brain tumour causing severe headaches and files a CI claim. The outcome should be consistent across all scenarios given that Sarah had the same pre-existing conditions in each case.

1. **Adviser A** (Full Disclosure):
 - Claim is likely to be paid in full.
 - The insurer has all relevant information about Sarah's history of migraines.
 - The full disclosure at the application stage means there's no issue of misrepresentation.
2. **Adviser B** (Encouraged Misrepresentation):
 - The insurer discovers the undisclosed migraine history during claim investigation.
 - Despite the non-disclosure, the claim would likely still be paid.
 - However, the insurer might:
 - Investigate whether the non-disclosure was careless or deliberate or innocent.
 - Consider applying a proportionate remedy (if not innocent), which could affect the claim amount.
 - The claim wouldn't be automatically declined, but the process might be more complicated and potentially result in a reduced payout.
3. **Comparison website** (Unintentional Non-disclosure):
 - Similar to Scenario 2, the insurer finds out about Sarah's undisclosed migraine history.
 - The claim would likely be paid, but the insurer might:
 - Seek clarification on why the information wasn't disclosed.
 - Consider whether the non-disclosure was innocent or careless.
 - Potentially apply a proportionate remedy, which could affect the claim amount.

Current Industry Practices

Current Industry Practices

Firm Termination

Call Listening

**“Good
Disclosure”
Programme**

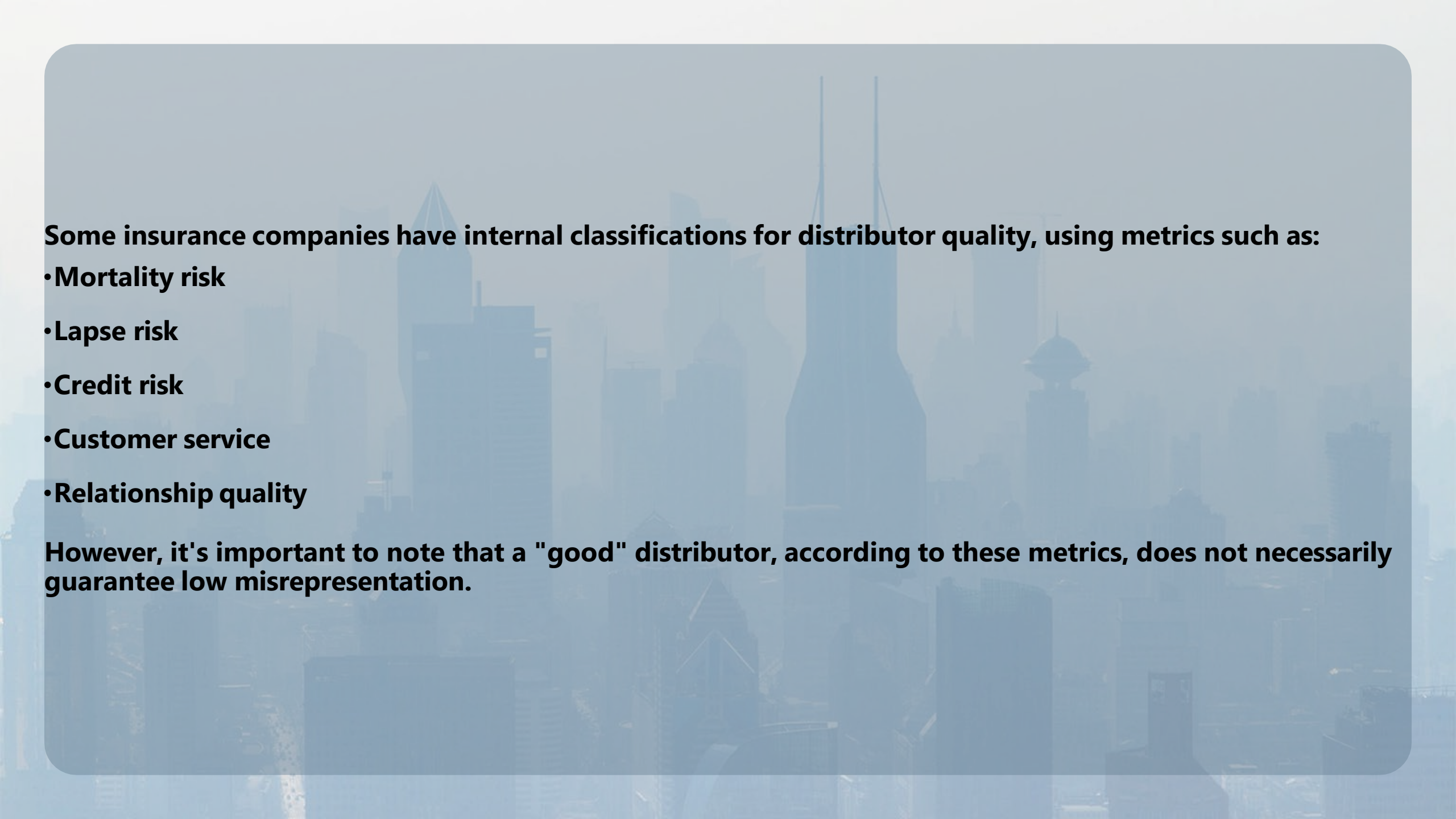
Site Visit

**Distributor-
specific Pricing**

**Post-Issue
Sampling**

**Disclosure
Modelling**

Claims Modelling

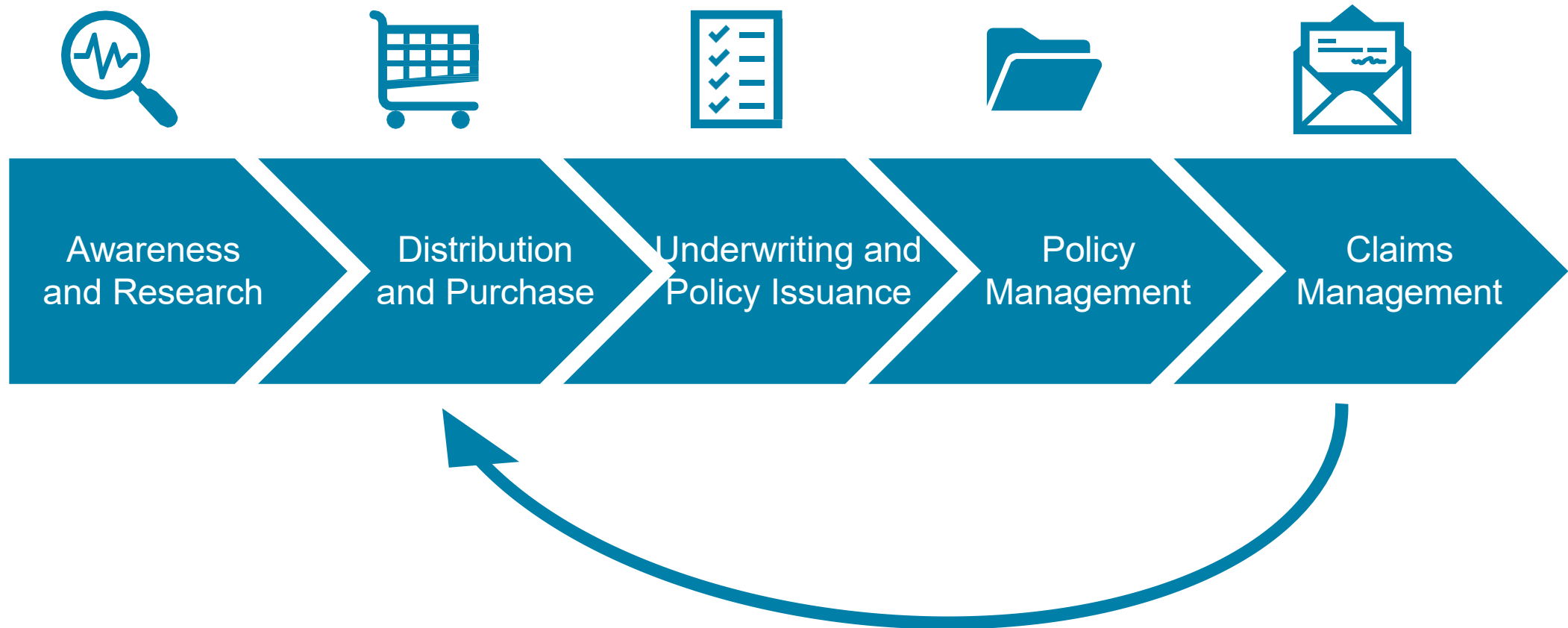
A faded, blue-tinted image of a city skyline, likely Chicago, serves as the background for the slide. The text is overlaid on this background.

Some insurance companies have internal classifications for distributor quality, using metrics such as:

- **Mortality risk**
- **Lapse risk**
- **Credit risk**
- **Customer service**
- **Relationship quality**

However, it's important to note that a "good" distributor, according to these metrics, does not necessarily guarantee low misrepresentation.

Insurance Customer Journey



Q&A



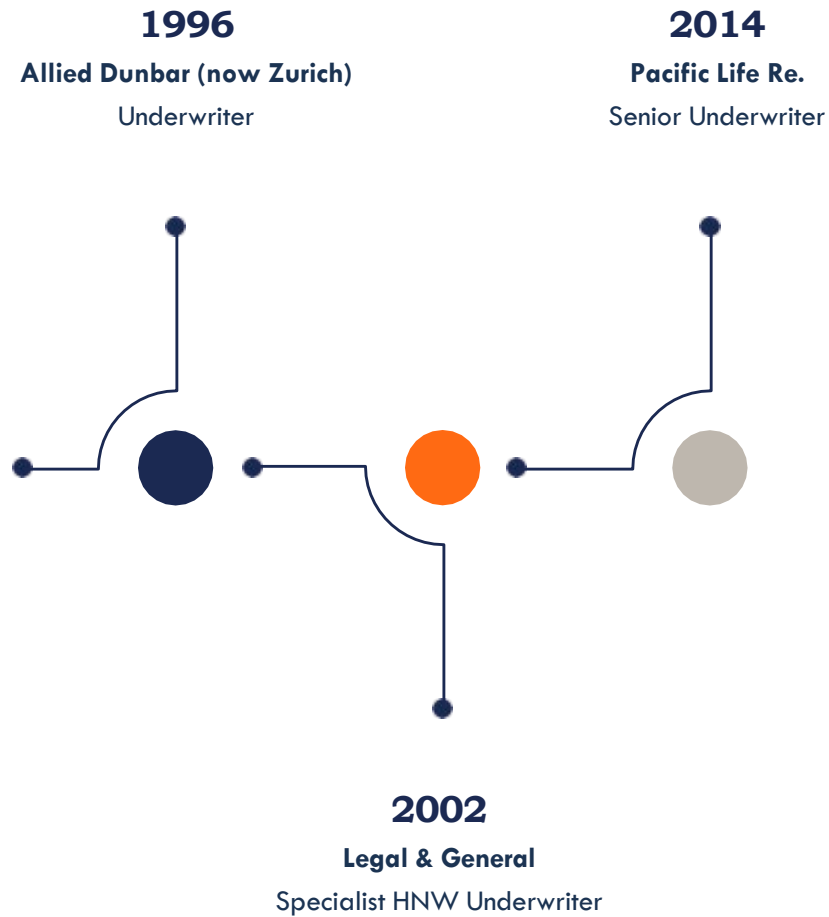
PACIFIC LIFE RE

Underwriting – Identifying Risks

HCF – 12 March 2025

Leighton Freeman

About me



Why underwrite?

Underwriters play a key role in safeguarding their office experience (and rates) by preventing avoidable claims through:-

- Identifying **anti-selective** purchase behaviour.
- Detecting **atypical** risks.
- Preventing **moral hazard** (e.g. murder, suicide, fraudulent claims).
- Obtaining the **right evidence** on atypical risks.

Application Form

Online:

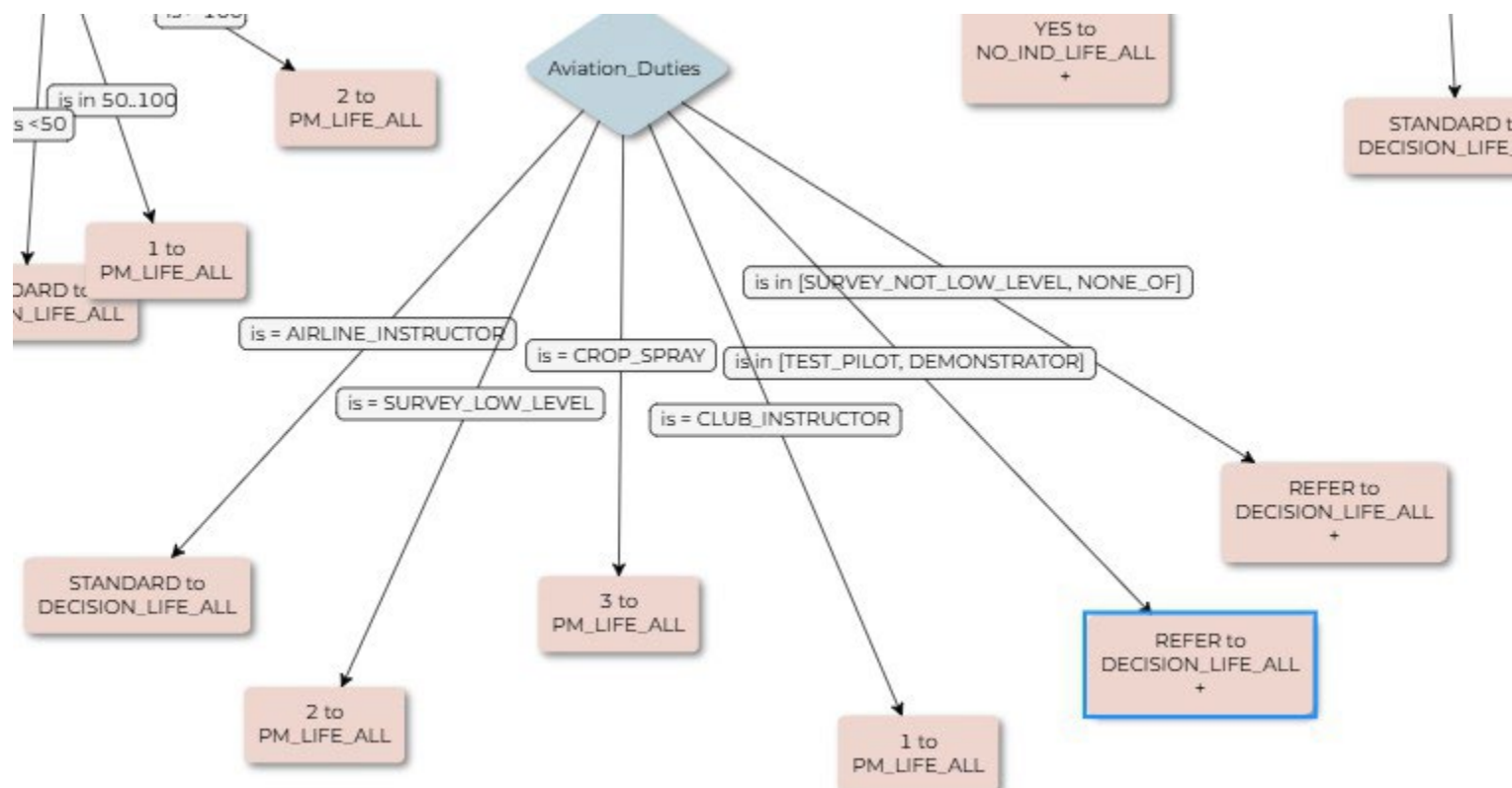
- **Ease of application.** Tailor questions to applicant/product etc.
- **Standardise disclosures.**
- **Use of linguistics.** Improve disclosure/engage customer.
- **Underwriting Rules Engine (URE).** Ask more questions about the disclosure.
- **Automation (STA/STP).** Leads to an underwriting decision (standard, rating/exclusion, postpone/decline), an evidence request or referral to an underwriter.
- **Improved MI.** Disclosure count, UW outcomes etc.
- **Protection Platform (by UnderwriteMe).** Comparison of fully underwritten outcomes from participating insurers.

Basis of contract

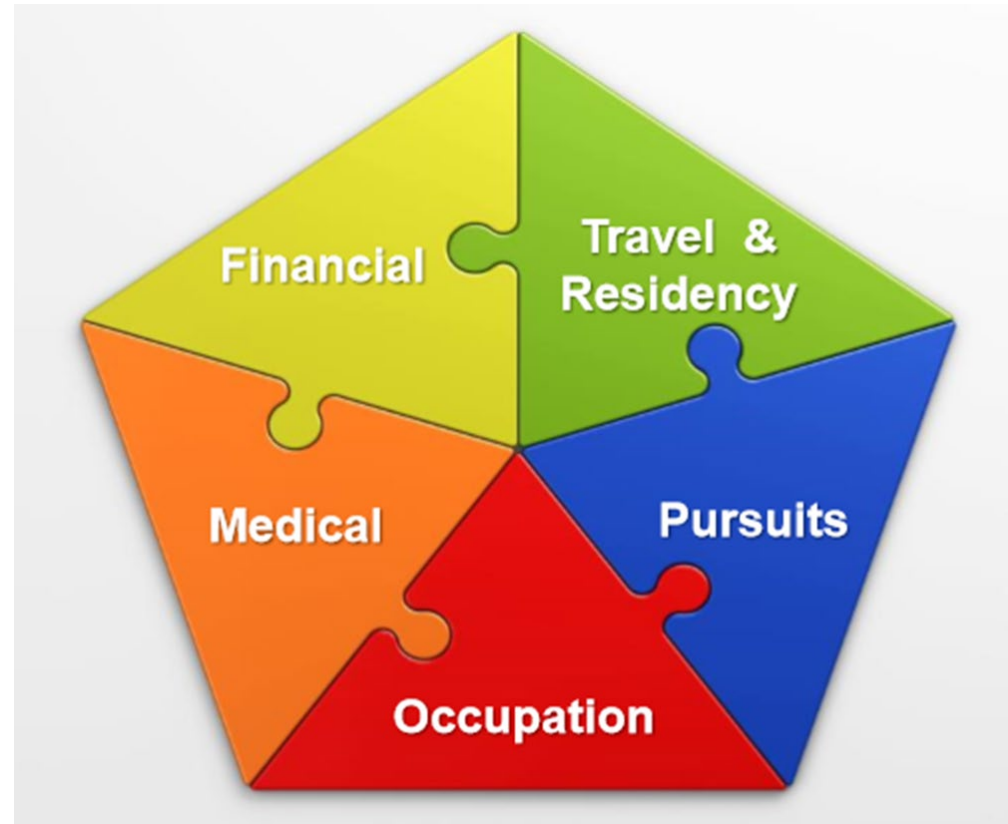
The answers provided by the customer form the basis of the contract of insurance.

Applications now typically contain a warning about failing to disclose accurately.

URE...example



Underwriting “jigsaw”



Pacific Life Re's 5 steps to getting it right

1. Pre-Underwriting Measures: Ask the right questions (app), obtain the right evidence (NMLs). Perform sanctions list checks.

2. Internet Due Diligence: Individual - name search, Linked-In, rich lists. Business – company website, Companies House (or Creditsafe, Riskdisk etc.).

3. Sanity Check: Does it make sense?

4. What's missing? Is additional evidence really necessary? What do we need to be able to accept the risk and why?

5. If not, why not? We need to be able to justify our actions if we restrict/decline cover.

Occupation



Armed Forces

- Under orders
- Aviation risks/Bomb disposal/Special Forces

Oil & Gas Industry

- Offshore
- Heavy machinery

Occupational duties

- Height work
- Driving
- Underground work

Travel/Residency

Eligibility:

- UK nationals predominantly living in the UK.
- Foreign nationals permanently resident in UK.
- **Some insurers**...Business cover on lives resident abroad where policy owned by UK company.
- **Some insurers**...UK/Foreign nationals permanently resident abroad with a UK liability.

Risk:

- Geopolitical situation.
- Access to healthcare.
- Potential for fraud/ability to administer claim.



Hazardous Pursuits



Participation in some hazardous pastimes can present extra risk.

Private Aviation
Mountaineering/Rock Climbing
Scuba diving
Skydiving

LIFE – per mille rating.

CIC/IP/TPD – exclude claims resulting from the activity.

Medical...Application Form

1

BMI & Habits

2

“Have you
ever...”

3

“In the last 5
years...”

4

Catch all
questions

5

Family History

1

BMI & Habits

Height & Weight
Smoking
Alcohol
Drugs

2

“Have you
ever...”

Cancer
Heart attack
Stroke
Diabetes
Neurological disorder
Severe Mental Health

3

“In the last 5
years...”

Gastro
Minor Mental Health
Eyes/Ears
Musculoskeletal

4

Catch all
questions

Anything you've not already told us about...
Referred to hospital...
Recent symptoms...
Medication/Treatment

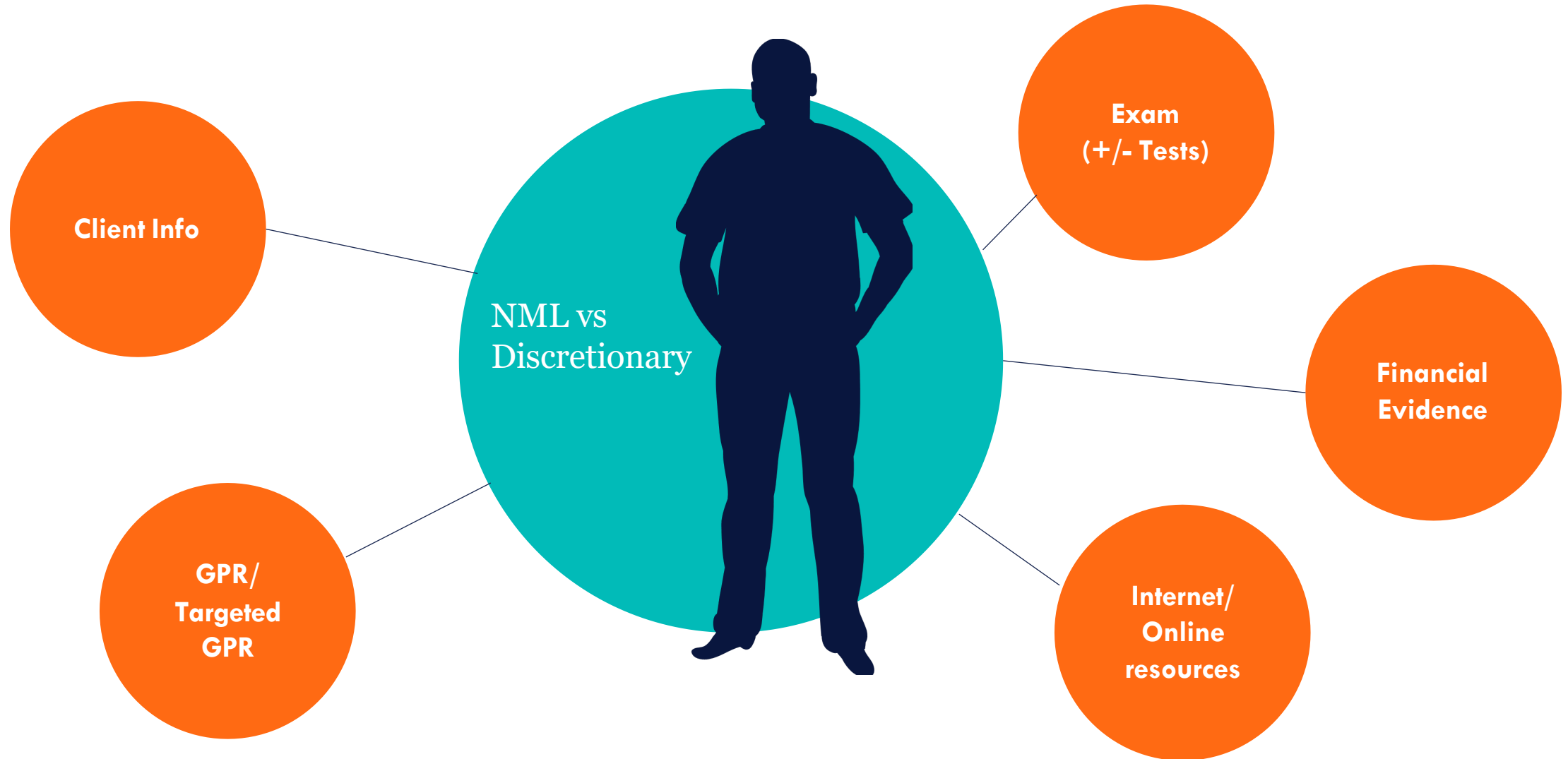
5

Family History

Ask about conditions that can/do run in families.
Not allowed to use genetic information...with 1 exception:
Huntingdon's disease (where life cover >£500k or CIC >£300k)



U/W Evidence





PACIFIC LIFE RE

Case Studies

Case Study 1



Application details:

- Application date 01/10/22
- £150k Life and CI (no TPD)
- Male age 53
- Scaffolder, heights up to 10 metres
- 6ft, 15st7lb = BMI 29
- Smoker 15CPD
- 10 UPW

Case Study 1- Underwriting



Evidence:

- None

Decision:

- Standard rates

Case Study 2

- **Application details:**
- Application date 10/11/23
- Start date 12/12/23
- £200k Life and CI (no TPD)
- Female age 36
- GP
- 5ft7, 10st7lb = BMI 23
- NS
- 12 UPW
- Anxiety, diagnosed October 2022, on medication, no time off work, no suicidal ideation/ thoughts of self harm, referred to Psychiatrist



Case Study 2- Underwriting

Evidence:

- Mental health TGPR
- Confirmed all disclosures on application form, referral was actually for 6 weeks of CBT which were completed

Decision:

- Standard rates



Thank you for listening!



PACIFIC LIFE RE

www.pacificlifere.com

LinkedIn

An aerial photograph of a speedboat moving across dark, choppy water. The boat is positioned in the upper right quadrant, leaving a wide, white, turbulent wake that curves and spreads outwards. The water's surface is textured with small waves and ripples. The overall color palette is dominated by dark blues and greys, with the white wake providing a sharp contrast.

SCOR

The Art & Science of Risk

HCF

Misrepresentation

01 Contract Law

02 Insurance History

03 Treating Customers Fairly

04 Consumer Insurance (Disclosure and Representations) Act 2012

05 Retrospective Underwriting

06 TCF Calls

Contract Law

Contract Law And Insurance

A contract (in law) is an agreement between at least two people –

One person agrees to undertake a certain act if / when the other person agrees to fulfil their side of the agreement

Such agreements are enforceable by LAW.

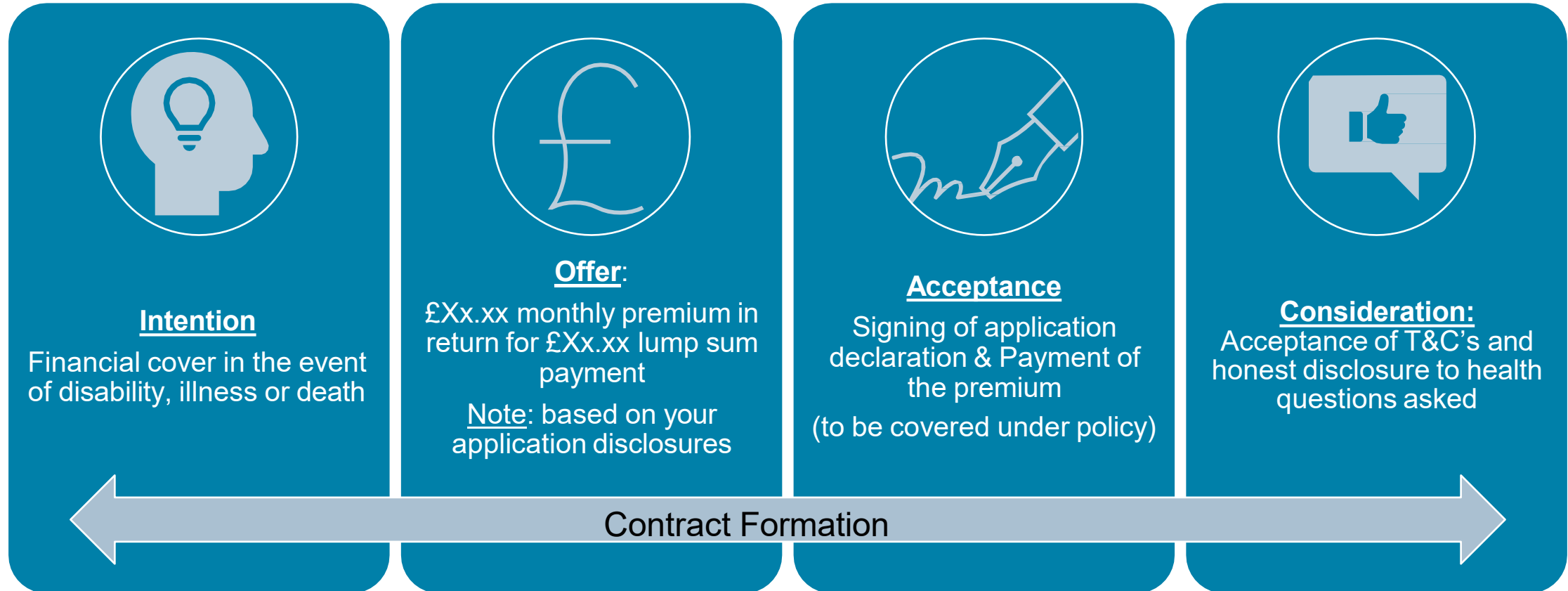
Enforceable contracts require some key provisions:

- Intention to create legal relationship
- Offer
- Acceptance
- Consideration

If any party does not fulfil their side of the agreement = Breach of contract and can avoid the agreement



Contract Law And Insurance



What is Misrepresentation

Misrepresentation – is an inaccurate or untrue statement made by one of the parties to the contract of insurance prior to the commencement of the policy



A History Lesson

We had a bad reputation - Headlines we rather forget

Insurer wouldn't pay my heart attack claim because I fainted 6 years ago

The Daily Telegraph

Claire paid health cover for years, but when she needed help, she was told she had 'the wrong type of cancer'

Daily Mail

I had open heart surgery and yet I didn't get my £100,000 because my Dad had the same op 15 years ago

the guardian

I've had a stroke but because I've had pins and needles a few times in the past, I'm no longer covered

Watchdog

Pay up you b*****s!!!!!!

**DAILY
STAR**

UK – The Old Approach

Case Study



Male Life, takes out Life, Critical Illness (CI) and Total Permanent Disability (TPD) – no disclosures, issued on Standard Rates

Claim 1 year into policy for Testicular Cancer (meeting the cancer definition)

Full medical records (FMR's) obtained as a standard requirement revealing a long history of back pain with occasional time off work, MRI scans, treatment with physiotherapy, chiropractor, pain management

Claims declined due to “**non-disclosure**”

UK – The Old Approach

Case Study – In hindsight



Seen as a breach of contract as Insurer did ask about back complaints – but retro UW would have only declined TPD and allowed CI and Life cover to commence

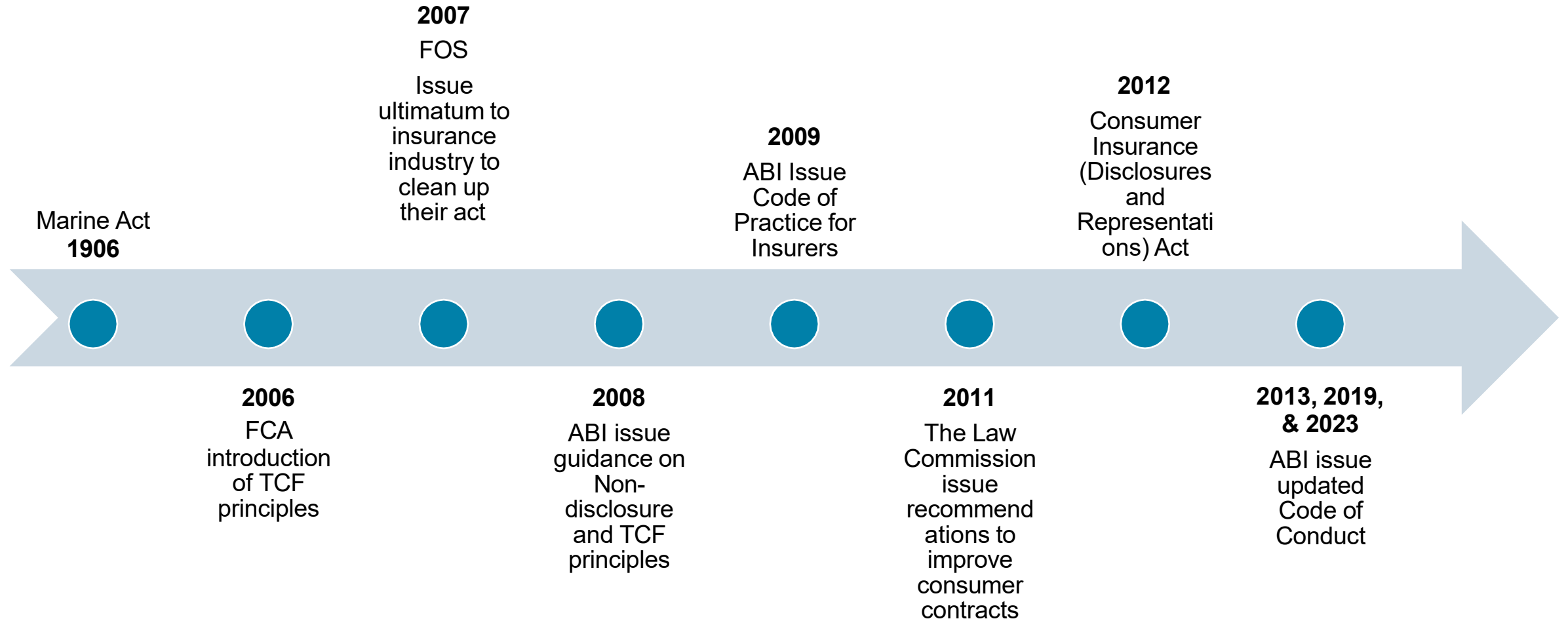
Insurers did not target evidence (often requesting FMRs as standard)

Reason for decline often poor – Claim declined due to “non-disclosure”

Communication often poor with decisions being sent via post out of the blue



Insurance Governance Over the years



Treating Customers Fairly (TCF)

The ABI Code of Practice

Treating Customers Fairly

Following evolving industry practice and FSA regulation around treating customers fairly (TCF), the Code sought to add clarification on what was considered fair treatment of claims beyond the legal guidance at the time

Introduced the categories of '**non-disclosure**'

- Innocent
- Negligent EDW
- Deliberate or Without any Care

Claims were at the forefront of consumer considerations:

- Collection of medical evidence
- Applications questions
- Sales process
- Proportionate remedies
- Consumer input prior to classification of non-disclosure

*The Code has since been routinely reviewed and updated in 2009, 2013 and 2019, 2023

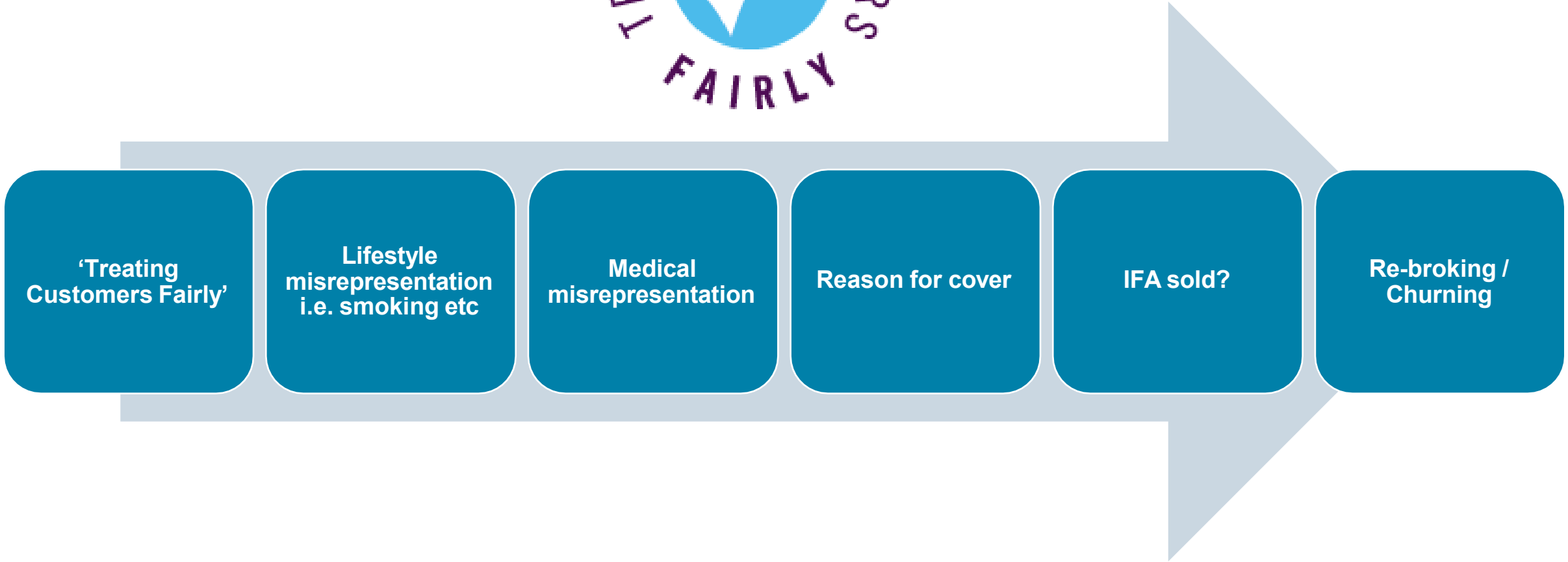
The ABI Code of Practice

Three Categories of Misrepresentation

Category	Explanation	Outcome
Innocent	• The customer has acted honestly and reasonably in all of the circumstances, including the customer's individual circumstances, but only where and to the extent these were known to the insurer. • In the circumstances, a reasonable person would have considered that the information was not relevant to the insurer. • The misrepresentation would have resulted in a different underwriting outcome.	Pay the claim in full
Careless	• Applies where the misrepresentation resulted from insufficient care – the failure by the customer to exercise reasonable care. This includes anything from an understandable oversight, or an inadvertent mistake, to serious negligence. • In the circumstances, a reasonable person would have considered that the information was relevant to the insurer. • The misrepresentation would have resulted in a different underwriting outcome.	Apply a proportionate remedy
Deliberate or reckless	• Only applies where the misrepresentation was deliberate or reckless. • In the circumstances, on the balance of probabilities, the customer knew, or must have known, that the information given was both incorrect and relevant to the insurer, or the customer acted without any care as to whether it was either correct or relevant to the insurer. • The misrepresentation would have resulted in a different underwriting outcome.	May avoid the policy (decline the claim and cancel the policy from inception)



Considerations



Consumer Insurance (Disclosure and Representations) Act 2012

Consumer Insurance (Disclosure and Representations) Act 2012 (aka CIDRA)

Why now?

To update the law relating to pre-contractual duties and simplify the legislation for consumers

CIDRA codified a lot of what had been put in place by the Code of Practice i.e. Insurers could remedy misrepresentation depending on 'careless' or 'deliberate' or 'reckless' (replacing 'negligent' and 'DoWac')

The Bill did change some aspects:

- Abolished the consumer's "duty to disclose" and replaced it with a duty "to take reasonable care not to make a misrepresentation"
- Consumers no longer expected to volunteer information the insurer deemed relevant ('duty of utmost Good faith); but
- Respond to questions asked by the insurer honestly & reasonably

Provided a new calculation to a new proportionate benefit calculation:

$$X = \frac{\text{Premium actually charged}}{\text{Higher premium}} \times 100$$

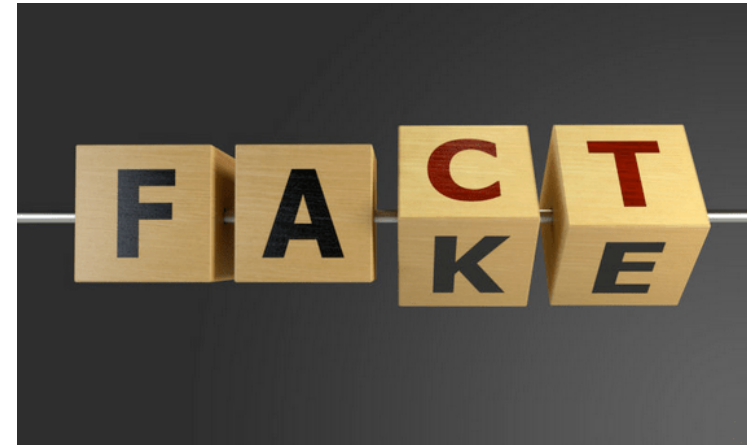
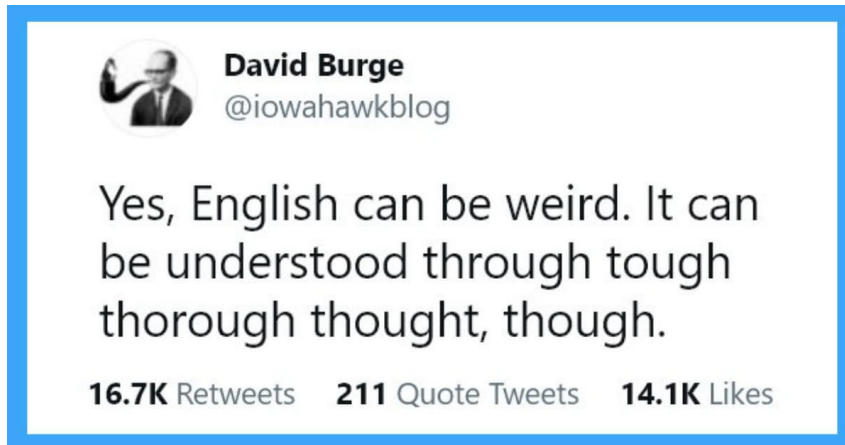
What is Material Misrepresentation

Material fact:

Industry practice is to only seek to pursue misrepresentation where the material fact would have led to a **rating**, **exclusion**, **postponement**, **declinature**, i.e. affected the underwriting terms offered. The Ombudsman would also take this approach.

Relevant Question:

The industry and the FOS would only expect Insurers to enforce misrepresentation where there are clearly understood and relevant questions





PACIFIC LIFE RE

Retrospective Underwriting and TCF calls

Health Claims Forum

Jennifer Taylor-Hill, Senior Claims Consultant

March 2025

What is Retrospective Underwriting and why do we do it?



In a nutshell....

- The primary reason for retro underwriting is to ensure that all parties are put back in the position that they would have been in, had the misrepresentation not occurred.

Retrospective Underwriting- for the Claims Assessor

The process:

- First you need to establish if the medical information you have found that wasn't disclosed is material.
- You then need to establish what the insured would or should have actually disclosed. Go back through the application questions applicable on *this contract*. Ideally list them out as part of your assessment
- Make sure you consider the duration the question is asking about, is it a 'have you ever' question or 'in the last 5 years' question for example?
- Think about the application process- what should have been disclosed to what question? What would have happened after that- drill down questions, or would medical information have been requested? (or both)
- If only drill down questions would have been asked, what information would this have elicited?
- Now you have established all the relevant medical information that can be used, copy this onto your referral sheet. This sheet should have already been completed with the full application details.
- Ask the Underwriter clear questions.
- Do not lead the Underwriter by disclosing to them the cause of claim

**Philosophy
Warning!**

Application questions



* How often have you been affected by these symptoms?

- ☐ A single episode only
- ☒ More than one episode
- ☐ Continual symptoms

* Have you had or been advised to have any of these investigations in the last 5 years? ⓘ

- ☐ MRI, CT or other scan of the brain
- ☐ Lumbar puncture
- ☐ Investigations to check the responsiveness of your muscles
- ☒ None of these

* What type of medical consultation or investigation have you had or are you awaiting regarding these symptoms? ⓘ

- ☐ Have seen GP or practice nurse only
- ☐ Have seen or been referred to a neurologist
- ☐ Have seen another specialist
- ☐ Have seen an ophthalmologist
- ☐ Recommended to see a medical practitioner but have not yet attended
- ☒ Not needed to see a doctor or nurse

Retrospective Underwriting- for the Underwriter

The process:

- Decisions should reflect the underwriting manual and philosophy at time of application.
- If appropriate, the underwriting manual page should be kept on file. We know that the FOS sometimes ask for this.
- The case should be underwritten based on all the medical information provided and, as best is possible, terms should be provided.

**Philosophy
Warning!**

Retrospective Underwriting- for the Underwriter

It's not always so straight forward:

- Are outstanding test results now known?

Can the UW make a decision knowing the result?

- Would the UW have postponed their decision?

How long?

Reconsider a new application?

- The UW would have wanted test results that are not available!

The insurer should apply expert judgement as to the terms that are most likely to have resulted

Does the claimants profile post application help with any 'on balance' opinion?

**Philosophy
Warning!**

Case Study 1



Details of claim obtained via tele-claim:

- Lung cancer Diagnosed 29/01/2025
- Initial symptoms- cough that had been on and off for six weeks and wouldn't clear. Went to see GP in November 2024 and was given Abx.
- Over Christmas 2024 started coughing up blood so went back to GP and was referred for Chest X-ray and then onwards under 2WW.

What medical information would you request?

Case Study 1-



Evidence;

Consultant report:

- Confirmed diagnosis and that definition was met

GPR:

- Past medical history of Asthma. Symptoms 2-3 days a week. Takes an inhaled bronchodilator and steroid inhaler. Had 5 days off work with it in the year prior to risk. FEV1 87%
- Raised BP mentioned in notes in January 2020, 160/93, told to change his diet, exercise and quit smoking. Come back in 6 months to check again. Nothing further in notes.
- Smokes 35 CPD, since in his 20's. Cessation advice given.

Application questions



Please tell us more about your Raised blood pressure

* Apart from your regular check-up, are you waiting for any of the following for your blood pressure?

- ☐ A hospital referral
- ☐ Tests or investigations
- ☐ The results of any tests or investigations
- ☒ None of these

* Have you had any of the following? ⓘ

- ☐ Kidney problems or protein in your urine
- ☐ Angina, a heart attack, a stroke, a TIA or blocked or narrowed arteries in your legs
- ☐ An ECG or heart test that was abnormal or needed further investigation
- ☐ Chest pain that required attendance at an Accident and Emergency department or any clinic or hospital
- ☐ Eye problems as a result of your condition
- ☒ None of these

* Are you taking prescribed treatment for your blood pressure?

- ☐ Yes
- ☒ No

* Have you ever stopped taking prescribed treatment without being told to do so by a doctor or nurse?

- ☐ Yes
- ☒ No

* Do you know your most recent blood pressure reading?

- ☒ Yes
- ☐ No

* On your most recent blood pressure reading, what was the higher number? ⓘ

160

* On your most recent blood pressure reading, what was the lower number? ⓘ

93

Case Study 1- Retro Example

POLICY DETAILS



Client	Insurance Company	Date of birth	01/04/1969
Treaty	LICI	Benefit	CI
Application Date	01/10/2022	Initial Sum Assured	£150,000.00
Start Date	01/10/2022	Initial Sum Reassured	£135,000.00
Disclosures and medical information at New Business	• Scaffolder, heights up to 10 metres • Smoker 15CPD • 10 UPW • 6ft, 15st7lb = BMI 29 • No FH		
Medical information at claims stage	• Asthma Dx age 10 • Tx bronchodilator and steroid inhaler • FEV1 August 2022 87% • 5 days off work in 2022 • No TOW 2021 • Smokes 35 CPD since early 20's- cessation advice given. • 10/01/2020- BP 160/93, lifestyle advice, no other entries		
Referral comments	• Please see above med info. Please can you confirm what the UWV decision would have been in 2022 if this information had been disclosed? Thanks.		



Case Study 1- Retro Example

Retro Decision.

Asthma since childhood. Good FEV1 of >80% but 2x Tx & 5 days off work 2022 are negative features (N.B. it's not uncommon for asthmatics to experience URIs requiring TOW, especially in winter months etc.) which would classify asthma as moderate.

Main issue is the heavier than declared smoking & the combination risk.

Moderate asthma +50% Life, +25% CIC.

If he was smoking >20cpd at time of application then additional +50% Life/CIC would apply.

So ...asthma looking 100% Life & 75% CIC.

^BP – clt was aged 50 at time BP reading (160/93) was taken. CVS calc +25% Life, +50% CIC based on that historic reading. Ideally, and especially for a substandard CIC risk, might have wanted up to date BP reading prior to offering terms however a rules engine might have offered with the rating? (NB CVS calc would uplift for >20cpd ... additional +25% Life, +50% CIC but a degree of double/triple counting so would only charge it once).

Terms could be possible +100-125% Life, +100-125% CIC.



Case Study 2

Details of claim obtained via tele-claim:

- Diagnosed with relapsing remitting MS in February 2025
- Initially saw GP with Symptoms of numbness and tingling in legs August 2024 and was reassured. Despite rest the symptoms returned.
- Went back to GP November 2024 and referred to Neurology.
- Diagnosed after Lumbar puncture and MRI8 in February 2025.

What medical information would you request?



Case Study 2-

Evidence;

Consultant report:

- Confirmed diagnosis and that definition was met
- When asked about symptoms, the consultant reported that the insured said in hindsight, symptoms had been on and off for around 2 years (since Jan 2023). She kept getting some numbness on the bottom of her right foot and pins and needles down her legs. She thought it was because she was training for a marathon so was related to this. However, it would always return even when she hadn't been exercising so much. By August 2024 she decided to see her GP.

GP report:

- An episode of blurred vision lasting for 5 days in December 2022. Went on its own, no further investigations or treatment.
- Showed first attendance with numbness and tingling August 2024, however no mention of a duration. She was reassured but reattended in November 2024 and was referred to Neurology at that point.



Application questions



Please tell us more about your Numbness and tingling

* Were you told that any of these are the cause of your symptoms?

- ☐ Multiple sclerosis
- ☐ Carpal tunnel syndrome
- ☐ Back problem
- ☐ Neck problem
- ☐ Joint problem
- ☐ Migraine
- ☐ Accident
- ☐ Injury
- ☐ Another medical condition
- ☒ None of the above

* Are you awaiting any of the following for this condition?

- ☐ Awaiting referral, tests, investigations, or investigation results
- ☐ Awaiting surgery
- ☒ No

* When did you first experience these symptoms?

01/01/2023

* Have you had any of the following symptoms in the last 5 years?

- ☐ Blurred or double vision or other visual disturbance
- ☐ Muscular co-ordination problems or clumsiness
- ☐ Tremor or involuntary shaking of the arms or legs
- ☐ Confusion or memory loss
- ☐ Dizziness, giddiness, unsteadiness, vertigo, loss of balance or fainting
- ☐ Slurred speech, a stutter, or other difficulty with speech or swallowing
- ☐ Difficulty with bladder or bowel control
- ☐ Drooping of the face
- ☒ None of these

Case Study 2- Retro Example

Client	Insurance Company	Date of birth	10/05/1987
Treaty	LICI	Benefit	CI
Application Date	10/11/23	Initial Sum Assured	£200,000.00
Start Date	12/12/23	Initial Sum Reassured	£180,000.00
Disclosures and medical information at New Business	• Occupation- GP • NS • 12 UPW • 5ft7, 10st7lb = BMI 23 • No FH • Anxiety Dx October 2022, On meds- 50mg Sertraline, no time off work, no suicidal ideation/ thoughts of <u>self harm</u>, no other Tx. TGPR Anxiety/ Depression: • Attended October 2022, anxious about events at work and work life balance, discussed mindfulness, wants to try medication and agreed. Start 50mg Sertraline. • November 2022- medication review no change, advised to come back next month before we increase. • January 23- medication review- feeling much better, calmer, wants to stay on meds. • Repeats ever since- no other consultations on record.		



Case Study 2- Retro Example

Medical information at claims stage	GP consultations; - December 2022- blurred vision R. eye 5 days, feels tired, been working hard- v busy home life, some pain when moves eye, discussed migraine? Stress? See how gets on next week and come back if doesn't resolve. May need referral Neuro/ Optician? - Jan 2023- Numbness bottom of R foot and pins and needles both legs. On and off.
Referral comments	- Please could you provide a UW decision based on the application form and the 1st bullet point of medical evidence. (blurred vision)? - Please could you also provide an opinion to include both bits of medical evidence? IE blurred vision and numbness, pins and <u>needles?</u> Thanks.



Case Study 2- Retro Example

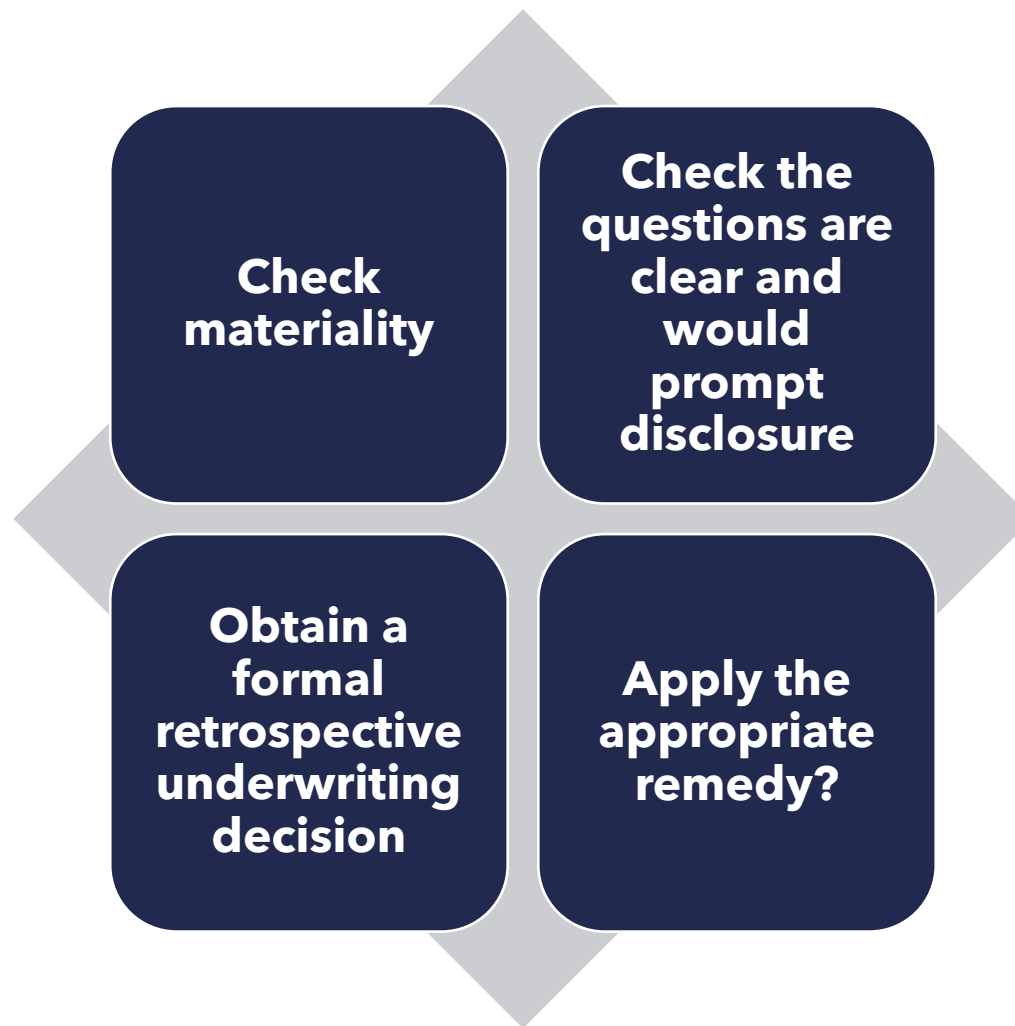
Retro Decision.

Episode of visual disturbance & eye pain could be suggestive of optic neuritis? Tiredness a non-specific symptom. The fact GP suggested may need further review & referral (potentially including neurology) would be a red flag. Number of differential diagnoses on the table and without any further knowledge of whether client continues to experience any issues (note client is a GP herself) would have been wary for CIC...unexplained and uninvestigated diplopia/blurred vision with last sx <2yrs would be postpone for CIC.

With the subsequent numbness/P&Ns the following month in >1 location in the context of age/sex and prior blurred vision...this would be enough to treat as possible MS and a +50-100% rating for life & CIC declined



Summary





PACIFIC LIFE RE

TCF calls

TCF Calls

Main Aim:

- This is the claimant's opportunity to explain why they answered the question(s) the way they did.
- Take the claimant back to the time of the application, explain why this matters
- Often customers will not understand why anything other than their current condition is relevant
- This is an opportunity for them to explain why there may be an innocent reason for their misrepresentation



Top Tips:

- Don't fear TCF calls, think of them as you would any other call with a customer such as an update call.
- Explain to the customer that you've noted some discrepancies in the medical evidence received versus what was advised at application and you need them to help you understand why they might not have advised of this history at the point of application when asked a relevant question.
- Prepare for your call- make sure you know your case, the key facts, specific dates and what was discussed with the customer at the time. Having a summary and/ or timeline of the misrep in front of you is useful.
- Listen to the customer- although you will have in mind the points you wish to cover off in your call, remember that you need to react and amend your conversation in response to the information the customer is providing.
- Don't be afraid to tell the customer that you need to go away and relook at the evidence in light of what they've advised (or suggest that they do this) and that you'll call them back again at a suitable time to reconvene.
- Don't be afraid to probe or question the customer more.
- If the customer contradicts what you're seeing in the medical evidence you should question them further on this. Discuss what you have in black and white in front of you.
- Don't lead the customer - rely on open questions. EG when discussing smoking it's best to simply ask the customer to tell you about their smoking habits over the years rather than simply asking about the 12 months pre-app.
- Don't ever assume- Some cases will be clear and calls will be short..... But there can always be a reason why!
- Silence is golden- don't feel you need to fill all silences.



TCF Calls – Tips & Tricks



Open-ended questions begin with:

- What
- Why
- How

Closed-ended questions begin with:

- Is /are
- Do / did
- Would / will
- Could / can
- Was / were
- Have / has
- Which
- Who
- When
- Where

Medical History Misrepresented

Post Application Signposts

Application Timings

Case Study 1-



Evidence:

GPR:

(Asthma)

- Past medical history of Asthma. Symptoms 2-3 days a week. Takes an inhaled bronchodilator and steroid inhaler. Had 5 days off work with it in the year prior to risk. FEV1 87%
- Raised BP mentioned in notes in January 2020, 160/93, told to change his diet, exercise and quit smoking. Come back in 6 months to check again.

(smoking)

- Smokes 35 CPD, since in his 20's. Cessation advice given.

Case study 1 (TCF call)

I was never told I had high blood pressure. I was told it was a little high and that I should make lifestyle changes

I wasn't given any medication and did not hear back from my GP

I told you I smoked and pay more for my premium for this. Why are you asking about this??

I don't remember how much I smoke. I use to smoke more when I was younger and did more manual labour on site. This days my role is more varied and I put that down on the application.

Oh yeah, my partner also made me cut lifestyle when the doctor told me to make life style changes

Case study 1 (TCF call)



Thoughts?

Innocent?

Careless?

Reckless/ Deliberate?

Case Study 2-

Evidence;

Consultant report:

- Symptoms had been on and off for around 2 years (since Jan 2023).
- She kept getting some numbness on the bottom of her right foot and pins and needles down her legs.
- She thought it was because she was training for a marathon so was related to this. However, it would always return even when she hadn't been exercising so much.
- By August 2024 she decided to see her GP.

GP report:

- An episode of blurred vision lasting for 5 days in December 2022. Went on its own, no further investigations or treatment.
- Showed first attendance with numbness and tingling August 2024, however no mention of a duration.



Case study 2 (TCF call)

I just put that down to being very stressed and busy and it went away, so I didn't do anything about it

Well, it wasn't really "blurred" vision, just a bit hazy, like I was tired.

I just put the pins and needles down to the shoes I was wearing. I was also training for a marathon

The pins and needles would usually go after a good night sleep, so I didn't think of it at the time of the application as there was no problem.

Why are you asking about this now!? I never suspected this was related to my diagnosis I'm claiming for now? This is very distressing.

Case study 1 (TCF call)



Thoughts?

Innocent?

Careless?

Reckless/ Deliberate?

Conclusions

Sales and distribution

Key to understanding basis of cover and sales quality

Underwriting

Risk is underwritten as a bigger picture assessment

Misrepresentation

Material if undisclosed medical history would have affected the underwriting terms

TCF

The claimant's opportunity provide additional context around medical evidence and can be categorised accordingly

WHAT DO ATTENDEES SAY ABOUT LUCID?

'This was my first LUCID event, and I'm absolutely determined to ensure it's not my last. I thought it was fantastic from registration to close; it was seamless.'

'Wonderful conference and atmosphere. Well done and look forward to 2025!'

'I loved the detailed presentations, the clear and efficiently run programme, the detail in the app and the chance to meet and chat with others.'

'I loved the diversity of subjects...it's all great education to help us when dealing with our customers and when designing new products.'



LIFE UNDERWRITING, CLAIMS AND INSURANCE DOCTORS

A collaborative conference organised by
representatives from all the UK & Ireland's industry clubs

